

B.2 PERFORMANCE WORK STATEMENT

Hematology Oncology Physician Services for VA Central California Health Care System

1. GENERAL:

- 1.1. Services Provided: The Contractor shall provide Board Certified /Board Eligible Hematology/Oncology Physician Services on site in accordance with the specifications contained herein to beneficiaries of the Department of Veterans Affairs (VA) and the VA Central California Health Care System, Fresno.
- 1.2. Place of Performance: Contractor shall furnish services at the *VA Central California Health Care System, Fresno, CA, 2615 E. Clinton Ave. Fresno CA 93703*
- 1.3. Authority: Title 38 USC 513 General Contracting Authority
- 1.4. Policy/Directives/Handbooks. The contractor shall be subject to the following policies, including any subsequent updates during the period of performance. The policies listed below can be accessed electronically at the following: [VA Publications](#) [VHA Publications](#)
 - 1.4.1. VA Directive 1663: Health Care Resources (HCR) Contracting – Buying Title 38 U.S.C. 815
 - 1.4.2. VHA Directive 1003.04: VHA Patient Advocacy
 - 1.4.3. VHA Directive 1065: Productivity and Staffing Guidance for Specialty Provider Group Practice
 - 1.4.4. VHA Directive 1088(1): Communicating Test Results to Providers and Patients
 - 1.4.5. VHA Directive 1100.18: Reporting and Responding to State Licensing Boards
 - 1.4.6. VHA Directive 1100.20: Credentialing of Health Care Providers
 - 1.4.7. VHA Directive 1100.21: Privileging
 - 1.4.8. VHA Directive 1192.01: Seasonal Influenza Vaccination Program for VHA Health Care Personnel
 - 1.4.9. VHA Directive 1220(1): Facility Procedure Complexity Designation Requirements to Perform Invasive Procedures in Any Clinical Setting
 - 1.4.10. VHA Directive 1605.01: Privacy and Release of Information
 - 1.4.11. VHA Directive 1907.01: VHA Health Information Management and Health Records
 - 1.4.12. VHA Handbook 1100.17: National Practitioner Data Bank (NPDB) Reports
 - 1.4.13. Privacy Act of 1974 (5 U.S.C. 552a) as amended:
http://www.justice.gov/oip/foia_updates/Vol_XVII_4/page2.htm
- 1.5. Acronyms/Definitions: Terms used in this contract shall be interpreted as follows unless the context expressly requires a different construction and/or interpretation. In case of a conflict in language between the Definitions and other sections of this contract, the language in this section shall govern.
 - 1.5.1. ACGME: Accreditation Council for Graduate Medical Education
 - 1.5.2. ACLS: Advanced Cardiac Life Support
 - 1.5.3. ASH: American Society of Hematology/Nephrology

- 1.5.4. BLS: Basic Life Support
- 1.5.5. CDC: Centers for Disease Control and Prevention
- 1.5.6. CEU: Certified Education Unit
- 1.5.7. Clinical Privileging: Clinical Privileging is the process by which a practitioner, licensed for independent practice; e.g., without supervision, direction, required sponsor, preceptor, mandatory collaboration, etc.; is permitted by law and the facility to practice independently, to provide specific medical or other patient care services within the scope of the individual's license, based upon the individual's clinical competence as determined by peer references, professional experience, health status, education, training and licensure. Clinical privileges must be facility-specific and provider-specific, and within available resources.
- 1.5.8. CME: Continuing Medical Education
- 1.5.9. CMS: Centers for Medicare and Medicaid Services
- 1.5.10. CO: Contracting Officer: The person executing this contract on behalf of the Government with the authority to enter into and administer contracts and make related determinations and findings.
- 1.5.11. COR: Contracting Officer's Representative: A person appointed by the CO to take necessary action to ensure the Contractor performs in accordance with and adheres to the specifications contained in the contract and to protect the interest of the Government. The COR shall report to the CO promptly any indication of non-compliance in order that appropriate action can be taken.
- 1.5.12. COS: Chief of Staff
- 1.5.13. CPARS: Contractor Performance Assessment Reporting System
- 1.5.14. Credentialing: Credentialing is the process of obtaining, verifying, and assessing the qualifications of a health care provider to provide care or services in or for the VA health care system. Credentials are documented evidence of licensure, education, training, experience or other qualifications.
- 1.5.15. DEA: Drug Enforcement Agency
- 1.5.16. EHR: Electronic Health Record - electronic health record system used by the VA
- 1.5.17. FSMB: Federation of State Medical Boards
- 1.5.18. FTE: Full Time Equivalent. VA's standard definition is for full time working the equivalent of 40 hours every week, 1,992 hours per year. However, providers may propose using their standard FTE definition.
- 1.5.19. HHS: Department of Health and Human Services
- 1.5.20. HIPAA: Health Insurance Portability and Accountability Act
- 1.5.21. ISO: Information Security Officer
- 1.5.22. Key Personnel: The individuals specified in this contract who are essential to work performance.

- 1.5.23. NPI: National Provider Identifier. NPI is a standard, unique 10-digit numeric identifier required by HIPPA. The Veterans Health Administration must use NPIs in all HIPAA-standard electronic transactions for individual (health care practitioners) and organizational entities (medical centers)
- 1.5.24. POP: Period of Performance
- 1.5.25. PPD: Purified Protein Derivative
- 1.5.26. PWS: Performance Work Statement
- 1.5.27. QA/QI: Quality Assurance/Quality Improvement
- 1.5.28. QASP: Quality Assurance Surveillance Plan
- 1.5.29. QM/PI: Quality Management/Performance Improvement
- 1.5.30. VETPro: is VHA's mandatory credentialing software platform to document the credentialing of VHA health care providers. This system facilitates completion of a uniform, accurate, and complete credentials file.
- 1.5.31. VHA: Veterans' Health Administration
- 1.5.32. VISN: Veterans Integrated Services Network
- 1.5.33. VISTA: Veterans Information Systems Technology Architecture

2. QUALIFICATIONS:

2.1. Staff/Facility

- 2.1.1. License: The Contractor's physician(s) assigned by the Contractor to perform the services covered by this contract shall have a current license to practice medicine in any State, Territory, or Commonwealth of the United States or the District of Columbia) when services are performed onsite on VA property.

All licenses held by the key personnel working on this contract shall be full and unrestricted licenses. Contractor's physician(s) who have current, full and unrestricted licenses in one or more states, but who have, or ever had, a license restricted, suspended, revoked, voluntarily revoked, voluntarily surrendered pending action or denied upon application will not be considered for the purposes of this contract.

- 2.1.2. Board Certification: All Contractor's physician(s) shall be Board Certified /Board Eligible Hematology/Oncology by the American Board of Internal Medicine (ABIM) in Hematology <http://www.abim.org/exam/certification/hematology.aspx>, and be currently certified in Basic Life Support (BLS), Advanced Cardiac Life Support (ACLS), or equivalency. All continuing education courses required for maintaining certification must always be kept up to date. Documentation verifying the Contractor's current certification shall be provided to the VA COR on an annual basis for each year of contract performance.
- 2.1.3. Credentialing and Privileging: Credentialing and privileging is to be done in accordance with the provisions of VHA Directive 1100.20 and VHA Directive 1100.21 referenced above. The Contractor is responsible for ensuring that the proposed physician(s) possess the requisite credentials enabling the granting of privileges. No services shall be provided by any Contractor's physician(s) prior to obtaining approval by the Facility Medical Executive Board and Medical Center Director.

- 2.1.3.1. If a Contractor's physician(s) and/or other contract provider(s) are not credentialed and privileged or has credentials/privileges suspended or revoked, the Contractor shall furnish an acceptable substitute without any additional cost to the government.
- 2.1.4. Technical Proficiency: Contractor's physician(s) shall be technically proficient in the skills necessary to fulfill the government's requirements, including the ability to speak, understand, read and write English fluently. Contractor shall provide documents upon request of the CO/COR to verify current and ongoing competency, skills, certification and/or licensure related to the provision of care, treatment and/or services performed. Contractor's physicians shall have knowledge of professional care theories, principles, practices, and procedures to perform assignments of Hematology patient/critically ill patient population. Contractor shall provide verifiable evidence of all educational and training experiences including any gaps in educational history for Contractor's physician(s) shall be responsible for abiding by the Facility's Medical Staff By-Laws, rules, and regulations (referenced herein) that govern medical staff behavior.
- 2.1.5. Continuing Medical Education (CME)/ Certified Education Unit (CEU) Requirements: Contractor shall provide the COR copies of current CMEs as required or requested by the facility. Contractor's physician(s) registered or certified by national/medical associations shall continue to meet the minimum standards for CME to remain current. Contractor shall report CME hours to the credentials office for tracking. These documents are required for both privileging and re-privileging. Failure to provide shall result in loss of privileges for contract physician(s).
- 2.1.6. Training (ACLS, BLS, EHR and VA MANDATORY): Contractor shall meet all VA educational requirements and mandatory course requirements defined herein; all training must be completed by the contractor's physician(s) as required by the VA. Other training may become required. VA will communicate any changes to the training requirement to the contractor. List all Training required here (TMS courses, EHR, etc.) and associated time. Remove this instruction from final RFP.

Training	Frequency (<i>once a year, etc.</i>)	Annual Hours
ACLS/BLS	<i>Annually</i>	<i>1</i>
Active Threat Training	<i>Annually</i>	<i>1</i>
Blood Administration: Complications	<i>N/A</i>	
EHR	<i>Annually</i>	<i>1</i>
Government Ethics	<i>As Needed</i>	<i>1</i>
Hospice and Palliative Care for VA Clinicians	<i>Annually</i>	<i>1</i>

Military Sexual Trauma (MST) for Medical Providers	<i>Annually</i>	<i>1</i>
Moderate Sedation In-Service Training	<i>N/A</i>	
PACT Act 2022 Toxic Exposure Screening (TES)	<i>Annually</i>	<i>1</i>
Patient Abuse	<i>Annually</i>	<i>1</i>
Patient Rights	<i>Annually</i>	<i>1</i>
Patient Safety	<i>Annually</i>	<i>1</i>
Prevention/Management of Disruptive Behavior/Violence Prevention Level I	<i>Annually</i>	<i>1</i>
Prevention of Workplace Harassment/No Fear Act	<i>Annually</i>	<i>1</i>
Suicide Prevention: Suicide Risk Management Training for Clinicians	<i>Annually</i>	<i>1</i>
SUX Infection Control and Blood Borne Pathogens	<i>N/A</i>	
VA Core Values Training (ICARE Recommitment)	<i>Annually</i>	<i>1</i>
VA Privacy and Information Security Awareness and Rules of Behavior	<i>Annually</i>	<i>1</i>
VHA MRI Safety Training Level 1 Training (all who enter MRI suites)	<i>N/A</i>	
VHA Privacy and HIPAA Focused Training	<i>Annually</i>	<i>1</i>
VISTA Imaging	<i>Annually</i>	<i>1</i>

2.1.7. **STANDARD INFECTION CONTROL MEASURES (PPD, IMMUNIZATIONS, ETC.):** Contractor shall provide proof of the following for physicians within five (5) calendar days after

contract award and prior to the first duty shift to the COR and Contracting Officer. **Tests shall be current within the past year.**

- 2.1.7.1. TUBERCULOSIS TESTING: Contractor shall provide proof of a negative Tuberculosis Skin Test (TST) or interferon-gamma release assays (IGRA) for all Contractor's physician(s) upon hire in accordance with CDC guidance. (This is applicable to all health care workers). A negative chest radiographic report for active tuberculosis shall be provided in cases of positive TST or IGRA results.
- 2.1.7.2. MEASLES, MUMPS, & RUBELLA TESTING: Contractors shall provide proof of immunity for all Contractor physicians {This is applicable to all health care workers}.
- 2.1.7.3. VARICELLA: Contractors shall provide proof of immunity for all Contractor physicians {This is applicable to all health care workers}.
- 2.1.7.4. ACELLULAR PERTUSSIS: Contractors shall provide proof of 1 dose of Tdap vaccination for all Contractor physicians {This is applicable to all health care workers}.
- 2.1.7.5. INFLUENZA: Contractors shall provide proof that all Contractor physicians have received the annual Influenza vaccine unless it is contraindicated. If the Contractor physician has a medical contraindication to the vaccine, they shall be required to wear a mask during the Influenza season. {This is applicable to all health care workers}.
- 2.1.7.6. OSHA REGULATION CONCERNING OCCUPATIONAL EXPOSURE TO BLOODBORNE PATHOGENS: Contractor shall provide evidence of completing and passing generic self-study blood-borne pathogen training for all Contractor's physician(s) {This is applicable to all health care workers}; provide their own Hepatitis B vaccination series and hepatitis B surface antigen test results following the hepatitis B vaccination series; maintain an exposure determination and control plan; maintain required records; and ensure that proper follow-up evaluation is provided following an exposure incident.
- 2.1.7.7. The facility shall notify the Contractor of any significant communicable disease exposures as appropriate. Contractor shall adhere to current CDC/HICPAC Guideline for Infection Control in health care personnel (as published in American Journal for Infection Control- AJIC 1998; 26:289-354 <http://www.cdc.gov/hicpac/pdf/InfectControl98.pdf>) for disease control. Contractor shall provide follow up documentation of clearance to return to the workplace prior to their return.
- 2.1.8. National Provider Identifier (NPI): NPI is a standard, unique 10-digit numeric identifier required by HIPAA. The Veterans Health Administration must use NPIs in all HIPAA-standard electronic transactions for individual (health care practitioners) and organizational entities (medical centers). The Contractor shall have or obtain appropriate NPI and if pertinent the Taxonomy Code confirmation notice issued by the Centers for Medicare and Medicaid Services (CMS) National Plan and Provider Enumeration System (NPPES) be provided to the Contracting Officer with the proposal.

- 2.1.9. DEA: Contractor shall provide a copy of the current DEA certificate.
- 2.1.10. Conflict of Interest: The Contractor and all Contractor's physician(s) are responsible for identifying and communicating to the CO and COR conflicts of interest at the time of proposal and during the entirety of contract performance. At the time of proposal, the Contractor shall provide a statement which describes, in a concise manner, all relevant facts concerning any past, present, or currently planned interest (financial, contractual, organizational, or otherwise) or actual or potential organizational conflicts of interest relating to the services to be provided. The Contractor shall also provide statements containing the same information for any identified consultants or subcontractors who shall provide services. The Contractor must also provide relevant facts that show how it's organizational and/or management system or other actions would avoid or mitigate any actual or potential organizational conflicts of interest. These statements shall be in response to the VAAR provision 852.209-70 Organizational Conflicts of Interest and fully outlined in response to the subject attachment in Section D of the solicitation document.
- 2.1.11. Citizenship related Requirements:
- 2.1.11.1. The Contractor certifies that the Contractor shall comply with any and all legal provisions contained in the Immigration and Nationality Act of 1952, As Amended; its related laws and regulations that are enforced by Homeland Security, Immigration and Customs Enforcement and the U.S Department of Labor as these may relate to non-immigrant foreign nationals working under contract or subcontract for the Contractor while providing services to Department of Veterans Affairs patient referrals;
 - 2.1.11.2. While performing services for the Department of Veterans Affairs, the Contractor shall not knowingly employ, contract or subcontract with an illegal alien; foreign national non-immigrant who is in violation their status, as a result of their failure to maintain or comply with the terms and conditions of their admission into the United States. Additionally, the Contractor is required to comply with all "E-Verify" requirements consistent with "Executive Order 12989" and any related pertinent Amendments, as well as applicable Federal Acquisition Regulations.
 - 2.1.11.3. If the Contractor fails to comply with any requirements outlined in the preceding paragraphs or its Agency regulations, the Department of Veterans Affairs may, at its discretion, require that the foreign national who failed to maintain their legal status in the United States or otherwise failed to comply with the requirements of the laws administered by Homeland Security, Immigration and Customs Enforcement and the U.S Department of Labor, shall be prohibited from working at the Contractor's place of business that services Department of Veterans Affairs patient referrals; or other place where the Contractor provides services to veterans who have been referred by the Department of Veterans Affairs; and shall form the basis for termination of this contract for breach.
 - 2.1.11.4. This certification concerns a matter within the jurisdiction of an agency of the United States and the making of a false, fictitious, or fraudulent certification may render the maker subject to prosecution under 18 U.S.C. 1001.

- 2.1.11.5. The Contractor agrees to obtain a similar certification from its subcontractors. The certification shall be made as part of the offerors response to the RFP using the subject attachment in Section D of the solicitation document.
- 2.1.12. Annual Office of Inspector General (OIG) Statement: In accordance with HIPAA and the Balanced Budget Act (BBA) of 1977, the Department of Health and Human Services (HHS) Office of Inspector General (OIG) has established a list of parties and entities excluded from Federal health care programs. Specifically, the listed parties and entities may not receive Federal Health Care program payments due to fraud and/or abuse of the Medicare and Medicaid programs.
- 2.1.12.1. Therefore, Contractor shall review the HHS OIG List of Excluded Individuals/Entities on the HHS OIG web site at <http://oig.hhs.gov/exclusions/index.asp> to ensure that the proposed Contractor's physician(s) are not listed. Contractor should note that any excluded individual or entity that submits a claim for reimbursement to a Federal health care program, or causes such a claim to be submitted, may be subject to a Civil Monetary Penalty (CMP) for each item or service furnished during a period that the person was excluded and may also be subject to treble damages for the amount claimed for each item or service. CMP's may also be imposed against the Contractor that employ or enter into contracts with excluded individuals to provide items or services to Federal program beneficiaries.
- 2.1.12.2. By submitting their proposal, the Contractor certifies that the HHS OIG List of Excluded Individuals/Entities has been reviewed and that the Contractors are and/or firm is not listed as of the date the offer/bid was signed.
- 2.2. Clinical/Professional Performance: The qualifications of Contractor personnel are subject to review by VA Medical Center COS or his/her clinical designee and approval by the Medical Center Director as provided in VHA Directive 1100.20 and VHA Directive 1100.21. Clinical/Professional performance monitoring and review of all clinical personnel covered by this contract for quality purposes will be provided by the facility COS and/or the Chief of the Service or his designee. A clinical COR may be appointed, however, only the CO is authorized to consider any contract modification request and/or make changes to the contract during the administration of the resultant contract.
- 2.3. Non- Personal Healthcare Services: The parties agree that the Contractor and all Contractor's physician(s) shall not be considered VA employees for any purpose.
- 2.4. Indemnification: The Contractor shall be liable for, and shall indemnify and hold harmless the Government against, all actions or claims for loss of or damage to property or the injury or death of persons, arising out of or resulting from the fault, negligence, or act or omission of the Contractor, its agents, or employees.
- 2.5. Prohibition against Self-Referral: Contractor's physicians are prohibited from referring VA patients to contractor's or their own practice(s).
- 2.6. Inherent Government Functions: Contractor and Contractor's physician(s) shall not perform inherently governmental functions. This includes, but is not limited to, determination of agency policy, determination of Federal program priorities for budget requests, direction and control

of government employees (outside a clinical context), selection or non-selection of individuals for Federal Government employment including the interviewing of individuals for employment, approval of position descriptions and performance standards for Federal employees, approving any contractual documents, approval of Federal licensing actions and inspections, and/or determination of budget policy, guidance, and strategy.

- 2.7. No Employee status: The Contractor shall be responsible for protecting Contractor's physician(s) furnishing services. To carry out this responsibility, the Contractor shall provide or certify that the following is provided for all their staff providing services under the resultant contract:

- 2.7.1. Workers' compensation
- 2.7.2. Professional liability insurance
- 2.7.3. Health examinations
- 2.7.4. Income tax withholding, and
- 2.7.5. Social security payments.

- 2.8. Tort Liability: The Federal Tort Claims Act does not cover Contractor or contract provider(s). When a Contractor or contract provider (s) has been identified as a provider in a tort claim, the Contractor shall be responsible for notifying their legal counsel and/or insurance carrier. Any settlement or judgment arising from a Contractor's (or contract physician(s)) action or non-action shall be the responsibility of the Contractor and/or insurance carrier.

- 2.9. Key Personnel:

- 2.9.1. The VA Full Time Equivalency (FTE) for the services required is 1.0 FTE for Hematology Oncology clinic services, including regular clinic hours of 1,992 (8hrs Mon-Fri,), no holiday, no on call or call back services.

Service	Hours Annually	FTE
Regular Clinic	1,992	1.0

- 2.9.2. The minimum number of Board Certified /Board Eligible Hematology Oncology physicians required to be on site daily is one as defined in paragraph Hours of Operation in this section.
- 2.9.3. The Contractor shall be responsible for providing coverage to the VA during periods of vacancies of the Contractor's personnel due to sick leave, personal leave, vacations and additional coverage as required. **In the event a scheduled physician is unable to complete an assigned shift, the contractor shall provide replacement physician coverage within 2 hours and notify the Contracting Office Representative (COR) immediately of the schedule change.**
- 2.9.4. Personnel Substitutions: During the first ninety (90) calendar days of performance, the Contractor shall make NO substitutions of key personnel unless the substitution is necessitated by illness, death or termination of employment. The Contractor shall notify the CO, in writing, within 15 calendar day(s) after the occurrence of any of these events and provide the information required below. After 90 days, the Contractor shall submit the information required below to the CO at least 15 calendar days prior to making any permanent substitutions.

- 2.9.4.1. The Contractor shall provide a detailed explanation of the circumstances necessitating the proposed substitutions, complete resumes for the proposed substitutes, and any additional information requested by the CO. Proposed substitutes shall have comparable qualifications to those of the persons being replaced. The CO will notify the Contractor within 15 calendar days after receipt of all required information of the decision on the proposed substitutes. The contract will be modified to reflect any approved changes of key personnel.
- 2.9.4.2. For temporary substitutions where the key person shall not be reporting to work for three consecutive workdays or more, the Contractor shall provide a qualified replacement for the key person. The substitute shall have qualifications comparable to those of the key person. Any period exceeding two weeks will require the procedure as stated above.
- 2.9.4.3. The Government reserves the right to refuse acceptance of any Contractor personnel at any time after performance begins, if personal or professional conduct jeopardizes patient care or interferes with the regular and ordinary operation of the facility. Breaches of conduct include intoxication or debilitation resulting from drug use, theft, patient abuse, dereliction or negligence in performing directed tasks, or other conduct resulting in formal complaints by patient or other staff members to designated Government representatives. Standards for conduct shall mirror those prescribed by current federal personnel regulations. Should the VA COS or designee show documented clinical problems or continual unprofessional behavior/actions with any Contractor's physician(s), s/he may request, without cause, immediate replacement of said Contractor's physician(s). The CO and COR shall deal with issues raised concerning Contractor's physician(s) conduct. The final arbiter on questions of acceptability is the CO.
- 2.9.4.4. Contingency Plan: Because continuity of care is an essential part of the Facility's medical services, The Contractor shall have a contingency plan in place to be utilized if the Contractor's physician(s) leaves Contractor's employment or is unable to continue performance in accordance with the terms and conditions of the resulting contract.

3. VA HOURS OF OPERATION/SCHEDULING

- 3.1.1. VA Business Hours: *VA Regular Work Hours: The VA regular work hours are Monday through Friday, from 8:00 am to 5:00 pm. A Hematology Oncology physician, either a contractor or a VA Hematology Oncology staff member, shall be on-site daily during regular work hours. This requirement is for 1.0 Equivalent (FTE) comprised of one (1) Certified /Board Eligible Hematology Oncology physician. The Contractor physicians shall provide five scheduled days, Monday through Friday, eight hours from 8 am to 4:30 pm, as agreed by the Chief of Medicine, COR, and CCFMG. No on-call or call back services outside regular workday hours. as agreed by the Chief of Medicine, COR, and CCFMG.*
- 3.1.2. Patients must be seen by a Contractor's physician(s) on-site at the facility in a timely manner in accordance with VA Rules and Regulations on clinic wait times and consult completion. Contractor shall notify the COR at least monthly about any obstacles to meeting this performance measure.

- 3.1.3. Contractor's physician(s) shall be available and present in clinic during normal facility clinic hours, which will be established, and may be revised, as deemed appropriate for patient care by the Chief of Staff. Currently, normal clinic hours are Monday through Friday, from 8:00 am to 5:00 pm.

3.1.3.1. Off-hours Coverage: none

- 3.2. Federal Holidays: The following holidays are observed by the Department of Veterans Affairs:

- New Year's Day
- President's Day
- Martin Luther King's Birthday
- Memorial Day
- Juneteenth
- Independence Day
- Labor Day
- Columbus Day
- Veterans Day
- Thanksgiving
- Christmas
- Any day specifically declared to be a national holiday.

- 3.3. Cancellations: *Unless a state of emergency has been declared (e.g. hospital is closed in an emergency, fire, etc.), the Contractor shall be responsible for providing services. VACCHCS will do its best to inform the contract physician of any changes in the schedule of needed services.*

4. **CONTRACTOR RESPONSIBILITIES**

- 4.1. Clinical Personnel Required: The Contractor shall provide the Contractor's physician(s) who are competent, qualified per this performance work statement, and adequately trained to perform assigned duties.
- 4.1.1. Contractor's physician(s) shall be responsible for signing in and out when in attendance. Time sheets will be used by the COR to confirm hours/day and services provided against the contractor's invoices.
- 4.2. Standards of Care: The contract physician(s)' care shall cover the range of Hematology services as would be provided in a state-of-the-art civilian medical treatment facility and the standard of care shall be of a quality, meeting or exceeding currently recognized TJC, VA and national standards as established by:
- 4.2.1. American Society of Hematology: <http://www.hematology.org/Clinicians/Guidelines.aspx>
- 4.2.2. The professional standards of the Joint Commission (TJC):
http://www.jointcommission.org/standards_information/standards.aspx

4.2.3. The standards of the American Hospital Association (AHA):
<http://www.hpoe.org/resources?show=100&type=8> and;

4.2.4. The requirements contained in this PWS

4.3. **MEDICAL RECORDS**

4.3.1. **Authorities:** Contractor's physician(s) providing healthcare services to VA patients shall be considered as part of the Department Healthcare Activity and shall comply with the 5 U.S.C.552a (Privacy Act), 38 U.S.C. 5701 (Confidentiality of claimants records), 5 U.S.C. 552 (FOIA), 38 U.S.C. 5705 (Confidentiality of Medical Quality Assurance Records) 38 U.S.C. 7332 (Confidentiality of certain medical records), Title 5 U.S.C. § 522a (Records Maintained on Individuals) as well as 45 C.F.R. Parts 160, 162, and 164 (HIPAA).

4.3.2. **HIPAA:** This contract and its requirements meet exception in 45 CFR 164.502(e), and do not require a BAA in order for Covered Entity to disclose Protected Health Information to: a health care provider for treatment of VA patients. Based on this exception, a BAA is not required for this contract. Health records generated by this contract or provided to the Contractors by the VA are covered by the VA Privacy Act system of records entitled 'Patient Medical Records-VA' (24VA10A7). Contractor generated VA Patient records are the property of the VA and shall not be accessed, released, transferred, or destroyed except in accordance with applicable laws and regulations. Contractor shall ensure that all records pertaining to medical care and services provided to VA patients are captured in the VA electronic health record system as required by VA policy as discussed in 4.4.4.
4.4.3.

4.3.3. **Disclosure:** Contractor's physician(s) may have access to patient medical records for the purpose of providing medical care and services to VA patients and performing services under the contract; however, Contractor shall obtain permission from the VA before disclosing any patient information outside VA. VA authorizes the Contractor to discuss patient health information for coordination of care with community health care providers in compliance with VA regulations, HIPAA and VHA Directive 1605.01, Privacy and Release of Information. The VA will provide the Contractor with a copy of VHA Directive 1907.01, Health Information Management and Health Records and VHA Directive 1605.1, Privacy and Release of Information. The penalties and liabilities for the unauthorized disclosure of VA patient information mandated by the statutes and regulations mentioned above, apply to the Contractor.

4.3.4. **Professional Standards for Documenting Care:** Care shall be appropriately documented in medical records in accordance with standard commercial practice and guidelines established by VHA Directive 1907.01 Health Information Management and Health Records: https://www.va.gov/vhapublications/ViewPublication.asp?pub_ID=9235 and all guidelines provided by the facility.

4.3.5. **Release of Information:** The VA shall maintain control of releasing any copies of patient health information or health records and will follow policies and standards as defined, but not limited to Privacy Act requirements. Contractor will not release or disclose copies of records and will refer all such requests to the Release of Information Department at the VA facility were assigned.

4.3.6. **Management for Medical Records:** National Archives and Records Administration record disposition requirements are found in RCS 10-1 Chapter 6, 6000 series.

4.4. Direct Patient Care: estimated 90% of the time is involved in direct patient care of the time involved in direct patient care.

4.4.1. Per the qualification section of this PWS, the Contractor shall provide the following staff:

- 4.4.1.1. Board Certified /Board Eligible Hematologist/ Oncologist: Evaluation and treatment, Contractor's physicians shall provide care to patients in critical/acute care settings in conformance with VA policies. Contractor's physicians shall assess, stabilize, and determine the disposition of inpatients with emergent conditions consistent with medical staff policy regarding emergency and consultative call services.
- 4.4.1.2. Discussion of end-of-life preferences/advanced care planning Consultations with sub-specialists.
- 4.4.1.3. Timely and proactive discharge planning, Coordination of care with the primary care physician
- 4.4.1.4. Discharge, including ensuring appropriate follow-up is arranged Perioperative care
- 4.4.1.5. The contractor's physicians shall provide care to patients in critical/acute care settings in conformance with VA policies. They shall also assess, stabilize, and determine the disposition of inpatients with emergent conditions consistent with medical staff policy regarding emergency and consultative call services. This includes running code teams and rapid response teams.
- 4.4.1.6. The core privileges in this specialty include the procedures on the list (below) and other procedures that are extensions of the same techniques and skills. This is not intended to be an all-encompassing list of procedures. It defines the types of activities/procedures/privileges that most practitioners in this specialty perform at the VA and inherent activities/procedures/privileges requiring similar skill sets and techniques.

Abscess incision and drainage

Removal of lines and tubes

4.4.2. Scope of Care: Contractor's physician(s) (as appropriate and within scope of practice/privileging) shall be responsible for providing Hematology care, including, but not limited to: Evaluation, Treatment and Management: Contractor's physicians shall admit and coordinate all aspects of care for hospitalized medical patients, including: Admission, including history and physical examinations, and Consultation Appropriate placement within the Hospital Provide Outpatient Clinical Services

- 4.4.2.1. Clinic and Surgical Care: Contractor's physician(s) shall provide clinical Hematology services. Contractor's physician(s) shall be present on time for any scheduled clinics as documented by physical presence in the clinic at the scheduled start time.

- 4.4.2.1.1. Contractor's physician(s) shall provide comprehensive clinical Hematology services. Typical procedures include, but are not limited to: Initiation, monitoring, and discontinuation of acute hemodialysis treatment, supplies and maintenance of all dialysis machines, reverse osmosis units, and dialysis supplies needed for dialysis treatments to include CRRT and standard renal replacement therapies.
- 4.4.2.1.1.1. Set up, operation, cleaning, and disinfecting of dialysis machine and related dialysis equipment IAW MTF policies and procedures.
 - 4.4.2.1.1.2. Attaching prescribed dialyzer and tubing to machine to assemble for use.
 - 4.4.2.1.1.3. Mixing of dialysate, according to formula using aseptic techniques.
 - 4.4.2.1.1.4. Priming dialyzer with saline or heparinized solution to prepare machine for use.
 - 4.4.2.1.1.5. Calculation of fluid removal or replacement to be achieved during dialysis procedure.
 - 4.4.2.1.1.6. Inspection of equipment settings, including pressures, conductivity (proportion of chemicals to water), and temperature to ensure conformance to safety standards.
 - 4.4.2.1.1.7. Initiation, termination, and monitoring of intradialytic treatments with Arteriovenous Fistula (AVF), Arteriovenous Graft (AVG), and catheters.
 - 4.4.2.1.1.8. Cleaning areas of access (fistula, graft, or catheter) and using antiseptic solution IAW MTF policies and procedures.
 - 4.4.2.1.1.9. Connecting of hemodialysis machine to access in patient's forearm or catheter site to start blood circulating through dialyzer.
 - 4.4.2.1.1.10. Ensuring hemodialysis machine is working properly and that blood is moving through the tubing.
 - 4.4.2.1.1.11. Inspection of venous and arterial pressures as registered on equipment to ensure pressures are within established limits.
 - 4.4.2.1.1.12. Monitoring of patient for adverse reaction and hemodialysis machine and other dialysis equipment malfunction throughout dialysis treatment in its entirety.
 - 4.4.2.1.1.13. Performance and documentation of patient's pre-, interim, and post-dialysis treatment review of patient condition. Recording of all treatment information. Documentation of other information related to the care of the patient in the individual patient record.
 - 4.4.2.1.1.14. Monitoring of patient status during dialysis treatment by assessing patient's vital signs. Documentation of dialysis treatment parameters on dialysis flow sheets used by the VAMC.
 - 4.4.2.1.1.15. Reporting any significant changes in patient condition to the COR.
 - 4.4.2.1.1.16. Checking blood chemistry deemed necessary to see if waste products are being removed via blood draws – pre and post dialysis treatment as ordered.

- 4.4.2.1.1.17. Preparation of lab specimens by placing patient identification label and specimen in biohazard bag for transport/delivery to lab.
- 4.4.2.1.1.18. Disinfecting water system; monitoring water quality, collecting water and dialysate samples, conducting air quality testing, maintaining accurate records, and documentation and follow up on procedures related to ensuring water quality per clinic protocol.
- 4.4.2.1.1.19. Cleaning and repair of equipment as needed and after each patient use.
- 4.4.2.1.1.20. Responsible for reporting any significant information relating to change in patient's condition or equipment problems to the COR.
- 4.4.2.1.1.21. Contractor personnel will ensure water quality control is maintained according to Association for the Advancement of Medical Instrumentation (AAMI) standards. This includes preventative maintenance according to manufacturer guidelines to include, but not limited to, periodic water sampling, and carbon and filter changes as manufacturer guidelines dictate. Duties may include:
 - 4.4.2.1.1.21.1. Disinfecting/flushing of dialysis and reverse osmosis machines.
 - 4.4.2.1.1.21.2. Heat/Chemical disinfection of dialysis machine.
 - 4.4.2.1.1.21.3. Monthly water sampling according to manufacturer guidelines.
 - 4.4.2.1.1.21.4. Changing carbon tank quarterly and as needed.
- 4.4.2.1.2. Postoperative Follow-Up. Contractor Provider rounds shall be conducted on postoperative patients in the Surgical Intensive Care Unit (SICU) and on the wards. All cases will be discussed in morbidity and mortality conferences, and the contractor physician(s) will provide appropriate information to the COR for inclusion in departmental reports.
- 4.4.2.1.3. Contractor's physician(s) shall provide consultative services at the patient's bedside if the patient is not ambulatory and in the clinic setting if the patient is able to report to the outpatient clinic. Procedures shall be scheduled for completion within 30 days of the date of the consult.
- 4.4.2.1.4. Mechanisms must be in-place to provide notification of test results for patients receiving care in accordance with VHA Directive 1088, Communicating Test Results to Providers and Patients.
 - 4.4.2.2. Medications: Contractor's physician(s) shall follow all established medication policies and procedures. No sample medications shall be provided to patients.
 - 4.4.2.3. Discharge education: Contractor's physician(s) shall provide discharge education and follow up instructions that are coordinated with the next care setting for all Hematology clinical patients.

4.4.3. **ADMINISTRATIVE**: estimated 5% of time not involved in direct patient

- 4.4.3.1. Quality Improvement Meetings: The Contractor's physician(s) shall participate in continuous quality improvement activities and meetings with

committee participation as required by the facility Chief of Service, Chief of Staff, or designee.

<i>Meeting</i>	<i>Frequency (once a year, etc.)</i>	<i>Annual Hours</i>
<i>QI meeting</i>	<i>N/A</i>	<i>N/A</i>

- 4.4.3.2. Staff Meetings: The Contractor's physician(s) shall attend staff meetings as required by the facility Chief of Service, Chief of Staff, or designee. Contractor to communicate with COR on this requirement and report any conflicts that may interfere with compliance with this requirement.

<i>Meeting</i>	<i>Frequency (once a year, etc.)</i>	<i>Annual Hours</i>
<i>Staff meeting</i>	<i>N/A</i>	<i>N/A</i>

- 4.4.3.3. QA/QI documentation: The Contractor's physician(s) shall complete the appropriate QM/PI documentation pertaining to all procedures, complications and outcome of examinations.

- 4.4.3.4. Patient Safety Compliance and Reporting: Contractor's physician(s) shall follow all established patient safety and infection control standards of care. Contractor's physician(s) shall make every effort to prevent medication errors, falls, and patient injury caused by acts of commission or omission in the delivery of care. All events related to patient injury, medication errors, and other breeches of patient safety shall be documented in the medical record of those impacted and disclosed to the patient or surrogate. As soon as practicable (but within 24 hours) Contractors shall notify COR of incident and submit an entry in the VA Patient Safety Reporting System, following up with COR as required or requested.

4.5. PERFORMANCE STANDARDS, QUALITY ASSURANCE (QA) AND QUALITY IMPROVEMENT(QI)

- 4.5.1. Quality Management/Quality Assurance Surveillance: Contractor's physician(s) shall be subject to Quality Management measures, such as patient satisfaction surveys, timely completion of medical records, and Peer Reviews. Methods of Surveillance: Focused Provider Practice Evaluation (FPPE) and Ongoing Provider Practice Evaluation (OPPE). Contractor performance will be monitored by the government using the standards as outlined in this Performance Work Statement (PWS) and methods of surveillance detailed in the Quality Assurance Surveillance Plan (QASP). The QASP shall be attached to the resultant contract and shall define the methods and frequency of surveillance conducted.
- 4.5.2. Patient Complaints: The CO will resolve complaints concerning Contractor relations with the Government employees or patients. The CO is final authority on validating complaints. If the Contractor is involved and named in a validated patient complaint, the Government reserves the right to refuse acceptance of the services of such personnel. This does not preclude refusal in the event of incidents involving physical or verbal abuse.

4.5.3. The Government reserves the right to refuse acceptance of any Contractor personnel at any time after performance begins, if personal or professional conduct jeopardizes patient care or interferes with the regular and ordinary operation of the facility. Breaches of conduct include intoxication or debilitation resulting from drug use, theft, patient abuse, dereliction or negligence in performing directed tasks, or other conduct resulting in formal complaints by patient or other staff members to designated Government representatives. Standards for conduct shall mirror those prescribed by current federal personnel regulations. The CO and COR shall deal with issues raised concerning the Contractor's conduct. The final arbiter on questions of acceptability is the CO.

4.5.4. Performance Standards

4.5.4.1. Measure: Provider Quality Performance

Performance Requirement: All Contractor's physician(s) shall perform in accordance with clinical standards.

Standard: OPPE documentation for all (100%) staff providing services under the contract. All staff (100%) meet Standards.

Acceptable Quality Level: 100% meet Standards

Surveillance Method: Ongoing Provider Performance Evaluation (OPPE) data pertinent to care performed for each provider working under this contract. OPPE data will review the following elements:

- A. Patient Care Performance
- B. Medical/Clinical knowledge
- C. Practiced Based Learning and Improvement
- D. Interpersonal and Communication Skills
- E. Professionalism
- F. System Based Practice

Frequency: Contractor will provide documentation for Ongoing professional practice evaluation (OPPE) and focused professional practice evaluation (FPPE)

4.5.4.2. Measure: Qualifications of Key Personnel

Performance Requirement: All Contractor's physician(s) shall be Board Certified /Board Eligible *Hematology Oncology physicians in accordance with the American Board of Internal Medicine's Hematology Standards.*

Standard: All (100%) Contractor's physician(s) are Board Certified /Board Eligible *Hematology Oncology physician* Acceptable Quality Level: 100%

Surveillance Method: Random Sampling of qualification documents

Frequency: Once a Month

4.5.4.3. Measure: Scope of Practice/Privileging

Performance Requirement: Contractor's physician(s) perform within their individual scopes of practice/privileging.

Standard: All (100%) Contractor's physician(s) perform within their scope of practice/privileges 100% of the time.

Acceptable Quality Level: 100% Contractor's physician(s) perform within their scope of practice/privileges 100% of the time.

Surveillance Method: Random Sampling of records.

Frequency: Bi-Weekly

4.5.4.4. Measure: Patient Access

Performance Requirement: The Contractor shall provide contract provider (s) in accordance with the operating hours and VA clinical schedule outlined in this PWS.

Standard: All (100%) Contractor's physician(s) are on time and available to perform services.

Acceptable Quality Level: Contractor's physician(s) are on time and available to perform services 98 % of the time.

Surveillance Method Random Sampling of Time and Attendance Sheets

Frequency: Weekly

4.5.4.5. Measure: Patient Safety

Performance Requirement: Patient safety incidents shall be reported using VA Patient Safety Reporting System. All incidents reported immediately (within 24 hours.)

Standard: All (100%) of patient safety incidents are reported using VA Patient Safety Reporting System within 24 hours of incident.

Acceptable Quality Level: 100% of patient safety incidents are reported using VA Patient Safety Reporting System within 24 hours of incident.

Surveillance Method: Random Sampling

Frequency: Once per Month

4.5.4.6. Measure: Maintains licensing, registration, and certification

Performance Requirement: Updated Licensing, registration and certification shall be provided as they are renewed. Licensing and registration information kept current.

Standard: All (100%) licensing, registration(s) and certification(s) for Contractor's physician(s) shall be provided as they are renewed. Licensing and registration information kept current.

Acceptable Quality Level: 100% licensing, registration(s) and certification(s) for Contractor's physician(s) shall be provided as they are renewed. Licensing and registration information kept current. No acceptable deviation.

Surveillance Method: Random Sampling

Frequency: Once per Month

4.5.4.7. Measure: Mandatory Training

Performance Requirement: Contractor shall complete all required training on time per facility policy

Standard: All (100%) of required training is complete on time by contract physician(s).

Acceptable Quality Level: 100%.

Surveillance Method: Random Sampling

Frequency: Once per Month

4.5.4.8. Measure: Privacy, Confidentiality and HIPAA

Performance Requirement: Contractor is aware of all laws, regulations, policies and procedures relating to Privacy, Confidentiality, and HIPAA and complies with all standards Zero breaches of privacy or confidentiality.

Standard: All (100%) contractor physician(s) comply with all laws, regulations, policies and procedures relating to Privacy, Confidentiality and HIPAA

Acceptable Quality Level: _100%

Surveillance Method: Contractor shall provide evidence of annual training required by the facility, report violations per VA Handbook 6500.6.

Frequency: Periodic Inspection

4.5.5. Registration with Contractor Performance Assessment Reporting System

4.5.5.1. As prescribed in Federal Acquisition Regulation (FAR) Part 42.15, the Department of Veterans Affairs (VA) evaluates Contractor past performance on all contracts that exceed the Simplified Acquisition Threshold and shares those evaluations with other Federal Government contract specialists and procurement officials. The FAR requires that the Contractor be provided an opportunity to comment on past performance evaluations prior to each report closing. To fulfill this requirement VA uses an online database, CPARS, which is maintained by the Naval Sea Logistics Center in Portsmouth, New Hampshire. CPARS has connectivity with the Past Performance Information Retrieval System (PPIRS) database, which is available to all Federal agencies. PPIRS is the system used to collect and retrieve performance assessment reports used in source selection determinations and completed CPARS report cards transferred to PPIRS. CPARS also includes access to the federal awardee performance and integrity information system (FAPIIS). FAPIIS is a web-enabled application accessed via CPARS for Contractor responsibility determination information.

4.5.5.2. Each Contractor whose contract award is estimated to exceed the Simplified Acquisition Threshold requires a CPARS evaluation. A government Focal Point will register your contract within thirty days after contract award and, at that time, you will receive an email message with a User ID (to be used when reviewing evaluations). Additional information regarding the evaluation process can be found at www.cpars.gov or if you have any questions, you may contact the Customer Support Desk @ DSN: 684-1690 or COMM: 207-438-1690.

4.5.5.3. For contracts with a period of one year or less, the contracting officer will perform a single evaluation when the contract is complete. For contracts

exceeding one year, the contracting officer will evaluate the Contractor's performance annually. Interim reports will be filed each year until the last year of the contract, when the final report will be completed. The report shall be assigned in CPARS to the Contractor's designated representative for comment. The Contractor representative will have sixty (60) days to submit any comments and re-assign the report to the CO.

- 4.5.5.4. Failure for the Contractor's representative to respond to the evaluation within those sixty (60) days, will result in the Government's evaluation being placed on file in the database with a statement that the Contractor failed to respond; the Contractor's representative will be "locked out" of the evaluation and may no longer send comments.

5. GOVERNMENT RESPONSIBILITIES

- 5.1. Contract Administration/Performance Monitoring: After award of contract, all inquiries and correspondence relative to the administration of the contract shall be addressed to: (enter contract administration if not already listed in another area- list the title (not name) and contact information for COR, Clinical point of contact, and any other relevant personnel involved).

5.1.1. CO RESPONSIBILITIES:

- 5.1.1.1. The Contracting Officer is the only person authorized to approve changes or modify any of the requirements of this contract. The Contractor shall communicate with the Contracting Officer on all matters pertaining to contract administration. Only the Contracting Officer is authorized to make commitments or issue any modification to include (but not limited to) terms affecting price, quantity or quality of performance of this contract.
- 5.1.1.2. The Contracting Officer shall resolve complaints concerning Contractor relations with the Government employees or patients. The Contracting Officer is final authority on validating complaints. In the event the Contractor effects any such change at the direction of any person other than the Contracting Officer without authority, no adjustment shall be made in the contract price to cover an increase in costs incurred as a result thereof.
- 5.1.1.3. In the event that contracted services do not meet quality and/or safety expectations, the best remedy will be implemented, to include but not limited to a targeted and time limited performance improvement plan; increased monitoring of the contracted services; consultation or training for Contractor personnel to be provided by the VA; replacement of the contract personnel and/or renegotiation of the contract terms or termination of the contract.

5.1.2. COR RESPONSIBILITIES:

The COR for this contract: Will be appointed through a COR Appointment Memo

- 5.1.2.1. The COR shall be the VA official responsible for verifying contract compliance. After contract award, any incidents of Contractor noncompliance as evidenced by the monitoring procedures shall be forwarded immediately to the Contracting Officer.

- 5.1.2.2. The COR will be responsible for monitoring the Contractor's performance to ensure all specifications and requirements are fulfilled. Quality Improvement data that will be collected for ongoing monitoring includes but is not limited to: Entering data that may be collected.
- 5.1.2.3. The COR will maintain a record-keeping system of services by Computer File. The COR will review this data monthly when invoices are received and certify all invoices for payment by comparing the hours documented on the VA record-keeping system and those on the invoices. Any evidence of the Contractor's non-compliance as evidenced by the monitoring procedures shall be forwarded immediately to the Contracting Officer.
- 5.1.2.4. The COR will review and certify monthly invoices for payment. If in the event the Contractor fails to provide the services in this contract, payments will be adjusted to compensate the Government for the difference.
- 5.1.2.5. All contract administration functions will be retained by the VA.

6. **SPECIAL CONTRACT REQUIREMENTS**

- 6.1. **Reports/Deliverables:** The Contractor shall be responsible for complying with all reporting requirements established by the Contract. Contractor shall be responsible for assuring the accuracy and completeness of all reports and other documents as well as the timely submission of each. Contractor shall comply with contract requirements regarding the appropriate reporting formats, instructions, submission timetables, and technical assistance as required.
 - 6.1.1. The following are brief descriptions of required documents that must be submitted by Contractor: upon award; weekly; monthly; quarterly; annually, etc. identified throughout the PWS and is provided here as a guide for Contractor convenience. If an item is within the PWS and not listed here, the Contractor remains responsible for the delivery of the item.

What	Submit as noted	Submit To
Quality Control Plan: Description and reporting reflecting the contractor's plan for meeting of contract requirements and performance standards	Upon proposal and as frequently as indicated in the performance standards.	Contracting Officer
Other than Cost and Price Information Supporting Proposed Physician Rate (required for Affiliate onsite hourly-remove if it does not apply)	Upon proposal, to submit EPA request, upon change in key personnel	Contracting Officer
Copy of Subcontracting Plan is required for all large businesses. Copy of Contractor Certification Statement if no subcontracting possibilities exist.	Upon proposal and as updated	Contracting Officer

Copies of any and all licenses, board certifications, NPI, to include primary source verification of all licensed and certified staff	Upon proposal and upon renewal of licenses and upon renewal of option periods or change of key personnel.	Contracting Officer with proposal; renewal submitted to VETPRO system.
Certification that staff list has been compared to OIG list	Upon proposal and upon new hires.	Contracting Officer
Proof of Indemnification and Medical Liability Insurance	Upon proposal and upon renewals.	Contracting Officer
Certificates of Completion for Cyber Security and Patient Privacy Training Courses	Before receiving an account on VA Network and annual training and new hires.	Contracting Officer
ACLS/BLS Certification	Upon award and every two years after award.	COR
Contingency plan for replacing key personnel to maintain services as required under the terms of the contract	Upon proposal and as updated	COR

6.2. Billing:

6.2.1. Invoice requirements and supporting documentation: Supporting documentation and invoice must be submitted no later than the 10 workday of the following month in which services were rendered. Subsequent changes or corrections shall be submitted by separate invoice. In addition to information required for submission of a “proper” invoice in accordance with FAR 52.212-4 (g), all invoices must include:

- 6.2.1.1. Name and Address of Contractor
- 6.2.1.2. Invoice Date and Invoice Number
- 6.2.1.3. Contract Number and Purchase/Task Order Number
- 6.2.1.4. Date of Service
- 6.2.1.5. Contractor’s physician(s) (*Name of Contractor’s employee*)
- 6.2.1.6. Hourly Rate
- 6.2.1.7. Quantity of hours worked
- 6.2.1.8. Total price

6.3. Vendor Electronic Invoice Submission Method: Invoices will be electronically submitted to the Tungsten website at <https://www.tungstenautomation.com/>. Tungsten Automation direct vendor support number is 877-489-6135 for VA contracts. The VA-FSC pays all associated transaction fees for VA orders. During Implementation (technical set-up) Tungsten Automation

will confirm your Taxpayer ID Number with the VA-FSC. This process can take up to 5 business days to complete to ensure your invoice is automatically routed to your Certifying Official for approval and payment. To successfully submit an invoice to VA-FSC please review “How to Create an Invoice” within the how to guides. All invoices submitted through Tungsten Automation to the VA-FSC should mirror the current submission of Invoice. Clarification of additional requirements should be confirmed with your Certifying Official (your CO or buyer). Payment will only be made for actual services rendered. Payments shall be made monthly in arrears. The VA-FSC requires specific information in compliance with the Prompt Pay Act and Business Requirements. The contractor shall be reimbursed upon receipt of a proper invoice. For additional information, please contact:

Tungsten Support

Phone: 1-877-489-6135

Website: <https://www.tungstenautomation.com/>

Department of Veterans Affairs Financial Service Center

<https://www.fsc.va.gov/einvoice.asp>

- 6.4. **Reduction in Services:** This is a fixed quantity contract for a specified number of hours. If, at the end of the period of performance, the government has not utilized the total number of hours required under this contract because of a change in its requirements, the parties agree that they will attempt to negotiate in good faith a contract modification reducing the scope of the contract with a corresponding adjustment in the total contract price. In no event will the VA pay for hours that exceed the total number of hours specified in the contract for the period of performance.
- 6.5. **Payments in full/no billing VA beneficiaries:** The Contractor shall accept payment for services rendered under this contract as payment in full. VA beneficiaries shall not under any circumstances be charged nor their insurance companies charged for services rendered by the Contractor, even if VA does not pay for those services. This provision shall survive the termination or ending of the contract.
- 6.5.1. To the extent that the Veteran desires services which are not a VA benefit or covered under the terms of this contract, the Contractor must notify the Veteran that there will be a charge for such service and that the VA will not be responsible for payment.
- 6.5.2. The Contractor shall not bill, charge, collect a deposit from, seek compensation, remuneration, or reimbursement from, or have any recourse against, any person or entity other than VA for services provided pursuant to this contract. It shall be considered fraudulent for the Contractor to bill other third-party insurance sources (including Medicare) for services rendered to Veteran enrollees under this contract.
- 6.6. Contractor Security Requirements (VA Handbook 6500.6)
- 6.7. **General Security Provisions:** *“Contractors, contractor personnel, subcontractors, and subcontractor personnel shall be subject to the same Federal laws, regulations, standards, and VA Directives and Handbooks as VA and VA personnel regarding information and information system security.”*

Access to VA Information & Systems: *“A contractor/subcontractor shall request logical (technical) or physical access to VA information and VA information systems for their employees, subcontractors, and affiliates only to the extent necessary to perform the services specified in the contract...” “All contractors, subcontractors, and third-party servicers and*

associates working with VA information are subject to the same investigative requirements as those of VA appointees or employees who have access to the same types of information. The level and process of background security investigations for contractors must be in accordance with VA Directive and Handbook 0710, Personnel Suitability and Security Program.” “Contract personnel who require access to national security programs must have a valid security clearance.” “The contractor/subcontractor must notify the Contracting Officer immediately when an employee working on a VA system or with access to VA information is reassigned or leaves the contractor or subcontractor’s employ. The Contracting Officer must also be notified immediately prior to an unfriendly termination.”

VA Information Custodial Requirements: “Information made available to the contractor or subcontractor by VA for the performance or administration of this contract... shall be used only for those purposes and shall not be used in any other way without the prior written agreement of the VA.” “VA information should not be co-mingled, if possible, with any other data on the contractors/subcontractor’s information systems or media storage systems in order to ensure VA requirements related to data protection and media sanitization can be met.” “If co-mingling must be allowed... the contractor must ensure that VA’s information is returned to the VA or destroyed in accordance with VA’s sanitization requirements.”

VA Information Custodial & Rights in Data: “Government shall receive unlimited rights to data/intellectual property first produced and delivered in the performance of this contract... This includes all rights to source code and all documentation created in support thereof.